

Policy on Confidentiality, Integrity and Security of Records, Data and Information

«Hospital Name»

Document control

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Prepared by	IT/MIS In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» maintains the confidentiality, integrity and security of records, data and information, protecting them from unauthorised access, tampering, loss or destruction.

1. Purpose

This policy sets out how «Hospital Name» maintains confidentiality of records, data and information; maintains their integrity; maintains their security; uses developments in technology to improve all three; discloses privileged health information only as authorised or required by law; and addresses requests for access to information in the medical records.

This policy does not cover medical record content, document control, or retention and destruction of records — those are covered in other hospital policies.

2. Scope

This policy applies to all records, data and information held by «Hospital Name», in physical and electronic form.

3. Policy standards

«Hospital Name» maintains the confidentiality of records, data and information.

«Hospital Name» maintains the integrity of records, data and information.

«Hospital Name» maintains the security of records, data and information.

«Hospital Name» uses developments in appropriate technology for improving confidentiality, integrity and security.

«Hospital Name» discloses privileged health information as authorised by the patient and/or as required by law.

«Hospital Name» addresses requests for access to information in the medical records by patients, physicians and other public agencies consistently.

4. Non-negotiable rules

1. Do not allow anyone other than authorised persons access to the contents of a record — access must align with applicable law.

2. Do not allow an entry in a record or in data to be tampered with, corrected, or overwritten outside the organisation's defined written guidance, and always keep a system to track changes made.
3. Do not leave records, data or information unprotected against loss or destruction — including pest/rodent control for physical records and virus/backup protection for electronic data.
4. Do not leave confidentiality, integrity and security technology unreviewed for improvement opportunities — including safeguarding patient data generated from medical equipment.
5. Do not disclose privileged health information without the patient's written authorisation or a legal requirement to do so.
6. Do not deny a patient, physician or public agency access to medical record information except where a licensed healthcare professional's opinion is that release would endanger the life or safety of patients or others.
7. Staff who see a rule under this policy broken report it the same shift to the IT/MIS In-Charge or the Medical Superintendent.

5. What we do

5.1 Confidentiality

«Hospital Name» ensures only authorised persons have access to the contents of a record, aligned with applicable law. Access to the medical records department and the Hospital Information System is controlled; electronic-system access differs by personnel type and is specific to that user. Physical records use a tracer card for tracking movement in and out of the medical records department. Records and data are not taken out of the areas where they are stored; electronic information cannot be copied at any location. Authentication, access control and automatic log-off features protect data privacy and security; ideally, only clinical care providers have access rights to a person's clinical records.

5.2 Integrity

«Hospital Name» ensures entries in records, data and information are not tampered with. Corrections are made only in accordance with the organisation's defined written guidance, which addresses how entries in the patient record are corrected or overwritten. «Hospital Name» maintains a system to track changes made in records or data.

5.3 Security

«Hospital Name» protects records, data and information against loss and destruction. Physical records are protected through adequate pest and rodent control measures, storage in fire-safe cabinets or areas with adequate and appropriate fire-fighting equipment. Electronic data is protected against viruses and trojans through a proper backup procedure and preventive measures against cyber-attacks. Confidential patient data — for example lab reports — is communicated through a secure messaging platform.

5.4 Technology for confidentiality, integrity and security

«Hospital Name» reviews and updates its technological features to improve confidentiality, integrity and security of information — for example moving from physical to electronic format, or remote backup of data. A mechanism safeguards patient data generated from medical equipment and its access to authorised people, including disabling USB ports of medical devices, restricting remote access and downloading of patient data, and regularly updating anti-virus software installed in medical equipment. Patient data backup from medical equipment is done on a timely basis. This may be based on findings from a risk assessment.

5.5 Disclosure of privileged health information

«Hospital Name» discloses privileged health information only as authorised by the patient — obtained in writing — and/or as required by law. Special care is taken in medico-legal cases and other special situations identified by the government and the organisation.

5.6 Access requests

Requests for access to information in the medical records — by patients, physicians and other public agencies — are addressed consistently at «Hospital Name». Release of information to patients follows the Code of Medical Ethics, 2002. Grievances concerning right-to-information requests are addressed by the government

and other applicable bodies, per written guidance. Denial of information is permitted only where, in the opinion of a licensed healthcare professional, release would endanger the life or safety of patients or others. Requests from physicians for access to records of patients they treated are addressed per the written guidance.

6. Governance and responsibility

Role	Responsibility
IT/MIS In-Charge	Owens this policy. Maintains access control, encryption/backup, and audit-trail systems.
Medical Records In-Charge	Controls physical access to the medical records department and manages tracer-card use.
Medical Superintendent	Oversees implementation of this policy. Decides on denial-of-information cases with clinical opinion.
All Staff	Follow access-control and confidentiality procedures. Report identified risks and rule breaches.

7. Quality monitoring

The IT/MIS In-Charge reviews access logs, backup/security incident records, and change-tracking logs. Documentary evidence is on file for each CORE objective element and each asterisked objective element (marked CORE and * respectively in the Level column of Section 12, Traceability table). This policy is reviewed annually, or sooner if NABH standards change.

8. Training and staff acknowledgement

All staff covered by this policy are trained on confidentiality, access control, and the disclosure/access-request procedures, and sign the acknowledgement below.

Acknowledgement statement:

"I have read the Policy on Confidentiality, Integrity and Security of Records, Data and Information of «Hospital Name». I will follow the processes described."

Signature table (one row per staff member):

Staff Name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy is distributed to the IT/MIS department, the Medical Records department, all department heads/unit in-charges, and is made available to all staff through the hospital intranet and department policy folders.

10. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
IMS	Information Management System (NABH Hospitals 6th Edition chapter)
OE	Objective Element

11. Traceability to NABH HCO Full Accreditation 6th Edition — IMS.5

This table is an index. It is not how the process is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
IMS.5.a	CORE*	Confidentiality of records, data, information maintained	Section 5.1	IT/MIS In-Charge
IMS.5.b	CORE	Integrity of records, data, information maintained	Section 5.2	IT/MIS In-Charge
IMS.5.c	CORE*	Security of records, data, information maintained	Section 5.3	IT/MIS In-Charge
IMS.5.d	Achievement	Technology used to improve confidentiality/integrity/security	Section 5.4	IT/MIS In-Charge
IMS.5.e	Commitment	Privileged health information disclosed as authorised/required	Section 5.5	Medical Superintendent
IMS.5.f	Commitment*	Access requests addressed consistently	Section 5.6	Medical Records In-Charge

12. Required Records / Evidence Checklist

IMS.5.a — Confidentiality.

- Access-control record for medical records department and Hospital Information System, by personnel type.
- Tracer-card usage record for physical file movement; authentication/log-off configuration record for electronic systems.

IMS.5.b — Integrity.

- Written correction/overwrite guidance document.
- Change-tracking log for records and data.

IMS.5.c — Security.

- Pest/rodent-control and fire-safety records for physical storage.
- Backup-procedure record, anti-virus/cyber-attack-prevention record, and secure-messaging-platform confirmation for confidential data.

IMS.5.d — Technology improvement.

- Technology-review and update record for confidentiality/integrity/security.
- Medical-equipment safeguard record — USB restriction, remote-access restriction, anti-virus update, timely data backup — and any supporting risk-assessment record.

IMS.5.e — Disclosure of privileged information.

- Written patient authorisation record for disclosure.
- Medico-legal/special-situation handling record.

IMS.5.f — Access requests.

- Written guidance for addressing patient/physician/public-agency access requests.
- Denial-of-information record with licensed-healthcare-professional opinion, where applicable.

13. References

- NABH Guidebook for Accreditation of Hospitals, 6th Edition — Standard IMS.5, "The organisation maintain confidentiality, integrity and security of records, data and information."
- National Medical Commission (adopting the Indian Medical Council). (2002). Professional Conduct, Etiquette and Ethics Regulations. <https://www.nmc.org.in/wp-content/uploads/2017/10/Ethics-Regulations-2002.pdf> — the current regulation named in IMS.5's own text for release of information to patients; the 2023 replacement regulations remain in abeyance and this 2002 regulation is the version currently in force.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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